

1. Administrative & Study Identification

Full Study Title: Open Label Phase 2 Study of Tisotumab Vedotin for Locally Advanced or Metastatic Disease in Solid Tumors **Short Title/Acronym:** innovaTV 207 **Protocol Number:** innovaTV 207 (18-312) **NCT Number:** NCT03485209 **Posting ID:** 766342340119063828 **Sponsor Name:** Pfizer (Seagen Inc., a wholly owned subsidiary of Pfizer, with Genmab) **Company:** Pfizer **Investigational Product:** Tisotumab vedotin (Tivdak) – tissue factor (TF)-directed antibody-drug conjugate **Phase of Development:** Phase II (PHASE2) **Trial Status:** ACTIVE_NOT_RECRUITING **Medical Monitor:** Pfizer/Seagen Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the activity, safety, and tolerability—specifically the investigator-determined confirmed Objective Response Rate (cORR) per RECIST v1.1—of tisotumab vedotin (TV) for the treatment of selected solid tumors. **Secondary Objectives:** To evaluate confirmed and unconfirmed ORR, disease control rate (DCR), duration of response (DOR), time to response (TTR), progression-free survival (PFS), overall survival (OS), safety, tolerability, pharmacokinetics, and immunogenicity.

2.2 Background & Rationale There remains a significant unmet medical need for patients with relapsed, locally-advanced, or metastatic solid tumors (including colorectal cancer, squamous non-small cell lung cancer [sqNSCLC], exocrine pancreatic adenocarcinoma, and head and neck squamous cell carcinoma [HNSCC]) who have experienced disease progression after standard therapies (e.g., platinum-based chemotherapy and immunotherapy). Tisotumab vedotin is a tissue factor-directed antibody-drug conjugate (ADC) that delivers a microtubule-disrupting agent (MMAE) to induce cancer cell apoptosis. This study aims to evaluate TV as both a monotherapy and in combination with immune checkpoint inhibitors (e.g., pembrolizumab) to improve clinical outcomes in these hard-to-treat populations.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Multi-cohort design: Parts A through G) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Targeted Enrollment: 352 participants **Number of Sites:** Global multicenter (including United States, Canada, France, Germany, Italy, Spain, United Kingdom) **Estimated Study Start:** June 2018 **Estimated Primary Completion:** March 2027

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Relapsed, locally-advanced or metastatic solid tumors (colorectal, pancreatic, sqNSCLC, or HNSCC) not candidates for standard therapy. 2. Disease progression on or after the most recent systemic therapy (e.g., for HNSCC: prior platinum-based regimen and/or checkpoint inhibitor). 3. Baseline measurable disease as measured by RECIST v1.1. 4. Eastern Cooperative Oncology Group (ECOG) Performance Status score of 0 or 1.
- 4.2 Exclusion Criteria** 1. Uncontrolled tumor-related pain, active brain metastasis, or peripheral neuropathy $\geq$ Grade 2. 2. Inflammatory lung disease or ongoing clinically significant toxicity associated with prior treatment. 3. Prior therapy with an anti-PD-1, anti-PD-L1, or anti-PD-L2 agent (specifically applicable to combination cohorts in Parts D, F, and G).
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** Tisotumab vedotin 1.7 mg/kg or 2.0 mg/kg administered on varying schedules based on the cohort (e.g., Days 1 and 15 of every 4-week cycle, or Days 1, 15, and 29 of every 6-week cycle). It is administered as monotherapy or combined with pembrolizumab $\pm$ carboplatin/cisplatin. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, investigator decision, or withdrawal of consent. A long-term extension phase (LTEP) is available for participants deriving continued clinical benefit.
- 5.2 Concomitant & Prohibited Therapy Permitted:** Standard oncology supportive care. Implementation of an appropriate eye care plan is strictly mandated to mitigate ocular adverse events. **Prohibited:** Concurrent investigational agents or other systemic anticancer therapies not specified in the study protocol.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging, PD-L1 testing, Ophthalmic Exam
Baseline	Cycle 1, Day 1	First Dose, Vitals, PK/PD, Safety Labs	Interim Cycles 2+ Efficacy Assessment (Every 6 weeks

for first 6 months, then Q12W), Safety Labs, Eye Care Monitoring
| | **End of Study** | Follow-up | Treatment Discontinuation,
Survival Follow-up, Subsequent Anticancer Therapy Tracking |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Investigator-determined confirmed Objective Response Rate (ORR) per RECIST v1.1.
Measurement Method: Tumor imaging (CT/MRI) central and/or local radiological assessment.

7.2 Secondary & Exploratory Endpoints Confirmed and unconfirmed ORR, Disease Control Rate (DCR), Duration of Response (DOR), and Time to Response (TTR). Progression-Free Survival (PFS) and Overall Survival (OS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous routine clinical observation. Special attention is directed to prespecified adverse events of special interest (AESIs) including ocular adverse events (OAEs), peripheral neuropathy, and bleeding events. **Serious Adverse Events (SAEs):** Evaluated continuously throughout treatment and during the protocol-defined post-treatment window. **Data Monitoring Committee (DMC):** Utilizes standard independent safety oversight across all trial cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Efficacy Set (treated patients with measurable disease at baseline) and Safety Set (all patients who receive ≥ 1 dose of TV). **Sample Size Justification:** Specific cohorts (e.g., 20 to 30 patients for targeted tumor types like HNSCC or sqNSCLC) are powered to estimate ORR with adequate precision. **Handling of Missing Data:** Standard oncology statistical methodologies, censoring time-to-event endpoints (PFS/OS) as per RECIST v1.1.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of trial data, efficacy assessments, and AESIs. **Audit Trail:** Maintained via verified eCRFs and central imaging repository logs.

11. Study Identifiers & Governance

Primary ID: innovaTV 207 **Posting ID:** 766342340119063828 **Registry Number:** NCT03485209 **Sponsor/Lead Organization:** Pfizer **Company:** Pfizer **Study Title:** Efficacy and Safety Study of Tisotumab Vedotin for Patients With Solid Tumors **Official Regulatory Title:** Open Label Phase 2 Study of Tisotumab Vedotin for Locally Advanced or Metastatic Disease in Solid Tumors **Trial Status:** ACTIVE_NOT_RECRUITING

12. Clinical Framework (Oncology Specific)

Primary Condition: Locally Advanced or Metastatic Solid Tumors (HNSCC, sqNSCLC, Pancreatic, Colorectal) **Preferred UMLS Name:** Solid Tumor **Disease Areas:** Colorectal Neoplasms, Carcinoma, Non-Small-Cell Lung, Exocrine Pancreatic Cancer, Carcinoma, Squamous Cell of Head and Neck **Diagnosis Threshold:** Confirmed disease progression following standard lines of systemic therapy **Medical Methodology:** Tissue Factor (TF)-directed antibody-drug conjugate (ADC) therapy alone or with CPIs **Optimization Target:** Confirmed Objective Response Rate (cORR)

13. Study Procedures & Methodology

Management Pathway: Oncology standard of care integrated with protocol-mandated, strict ocular care mitigation plan **Education Protocol (HCP):** Site initiation and ongoing training focusing on ADC toxicities (OAEs, peripheral neuropathy, and bleeding) **Education Protocol (Patient):** Informed consent, treatment scheduling, and mandatory prophylactic eye care instructions **Quality Audit Mechanism:** Regular local/central RECIST v1.1 radiological reviews and safety reporting audits

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression, unacceptable toxicity, or LTEP completion (Expected trial lifecycle 2018–2027) **Onsite Visit Cadence:** Cycle Days 1, 15 (and 29 for 6-week cycles) **Remote Monitoring Frequency:** Routine AE surveillance; tumor assessments evaluated Q6W for the first 6 months, then Q12W **Checklist Review Interval:** Safety, labs, and concomitant medication assessment at every treatment cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				

Clinical Operations Lead | [Name] | ____ | [DD-MMM-YYYY] | | Lead
Statistician | [Name] | _____ | [DD-MMM-YYYY] |